

Scope of Work by Claude

REF: <https://claude.ai/chat/5fe3a5c5-ca67-4f8f-bfce-f388929de304>
REF2: <https://chatgpt.com/g/g-p-6a27047216788191b6530a921dc45659-credentialing-revision/c/6a2704f4-622c-83ea-a841-7e37629bc2dc>

USRad-Addendum-B-PDF-Revision-Spec (1) Addendum B: PDF and Provider Page Revision Spec

August 3, 2026 | Internal Document | Not for Distribution

Exact replacement copy for the two downloadable PDFs and the affected provider component. Decisions settled in discussion; this is the build sheet.

SETTLED ECONOMICS

The chain, reconciled:

	Value
Center per-scan reimbursement, plain MRI	\$225–\$400, varies by market
USRad fee, flat per modality	\$75 (plain MRI)
Published global price to patient or employer	\$300–\$475
Volume basis	15–25 additional scans/month
Realistic monthly add to center	\$3,400–\$10,000

Every figure in this spec derives from these five lines. Nothing else in either document may state per-scan or monthly economics independently.

Rules now in force:

1. **"Per-scan reimbursement," never "net margin."** USRad cannot know a center's net margin. It knows what it pays.
2. **Plain MRI only.** No "avg across modality mix" language until the full flat-fee table is finalized.
3. **15–25 scans/month is the single volume basis.** No component may imply a different one.
4. **The incremental argument goes in prose, not in a number.**

| 1. PROVIDER OPPORTUNITY BRIEF

| 1.1 Page 1 — revenue tiles

Replace all three tiles and the footnote.

Tile	Current	Replace with
Left	\$375 / NET MARGIN PER SCAN / avg across modality mix	\$225–\$400 / PER-SCAN REIMBURSEMENT / plain MRI, varies by market
Center	+15–25 / ADDITIONAL SCANS/MO / from existing capacity	<i>unchanged</i>
Right	\$5K–\$10K / REALISTIC MONTHLY ADD / at 15–25 scans/month	\$3,400–\$10,000 / REALISTIC MONTHLY ADD / at 15–25 scans/month

Footnote — replace in full:

* Per-scan reimbursement reflects contracted rates for plain MRI and varies by market. Each center sets its own rate, subject to USRad approval, and receives that rate on every assignment. Monthly figures assume 15–25 additional scans per month. Results vary.

Removes the modality-mix claim, states that the center sets the rate, and gives the range a stated basis.

| 1.2 Page 1 — the incremental margin argument

Add below the tile row, as prose rather than a figure:

Why incremental volume matters. When a scanner has open capacity, an additional completed scan adds revenue without a matching increase in equipment, occupancy, or administrative overhead. Each center can evaluate the contribution against its own read fees, staffing, and supply costs.

This is the argument the \$375 figure was trying to make. In prose it is verifiable by the center against its own numbers and cannot be arithmetically falsified.

Revised from an earlier draft that read "your fixed costs are already covered... the tech time, equipment, and overhead are already paid for whether the slot runs or sits empty." That version asserted a cost structure USRad cannot see. Tech time is not reliably fixed where overtime, per diem, or flexed staffing is involved, and not every center's fixed costs

are covered in every operating period. The revised language makes the same argument without stating the center's economics for them — and inviting the center to run its own numbers is more persuasive than telling them the answer.

1.3 Page 1 — headline

Current	Issue	Replace with
Turn Empty Scanner Slots Into Guaranteed Monthly Revenue.	"Guaranteed" attaches to <i>monthly revenue</i> , implying a volume commitment. Contradicts FoundingPartners.astro:327 — "No volume commitments"	Turn Empty Scanner Slots Into Predictable Revenue.

The 10-day payment guarantee is defensible and stays. A revenue guarantee is not.

1.4 Page 1 — subhead

Current	Replace with
USRad delivers pre-paid, price-confirmed patients directly to your imaging center. No insurance. No billing complexity. Payment issued within 10 days of fulfillment.	USRad delivers pre-funded, price-confirmed assignments directly to your imaging center. No insurance. No billing complexity. Payment issued within 10 business days of fulfillment.

Also corrects "10 days" to "10 business days," matching the guarantee box below it and PSA § 4.3.

1.5 Page 1 — guarantee box

Line	Current	Action
1	Payment issued within 10 business days of scan fulfillment.	Keep
2	Every assignment represents a patient who has already agreed to the price.	Replace: Every assignment is funded before it is routed to your center.
3	No denials, no chargebacks, no collections. Revenue is confirmed before the patient arrives.	Revise: No insurance denials, no patient collections, no claims follow-up. Funding is confirmed before the patient arrives.

Why "chargebacks" comes out. It is a card-industry term covering a broader set of events than insurance adjudication. Patient card disputes, fraud reversals, duplicate

payments, and refunds remain possible under any model, and USRad cannot promise otherwise. "No insurance denials, no patient collections, no claims follow-up" states the actual benefit precisely and does not make an absolute treasury claim. "Revenue" also becomes "funding," since the sentence is about the assignment being funded rather than about the center's revenue recognition.

1.6 Page 1 — section header and bullets

Current	Replace with
WHY CASH-PAY IMAGING WORKS FOR YOUR CENTER	WHY PRE-FUNDED IMAGING WORKS FOR YOUR CENTER

Bullets: all eight are funding-neutral and stay as written.

1.7 Footer — both pages

Current	Replace with
usrad.com U.S. Radiology of Florida LLC National Cash-Pay Imaging Infrastructure	usrad.com U.S. Radiology of Florida LLC National Pre-Funded Imaging Infrastructure

Appears on every page directly under the legal entity name. Strongest instance of cash-pay-as-identity in any artifact.

1.8 Page 2 — portal flow, Step 1

Six of seven steps are funding-neutral and stay as written. Step 1 only:

Current	Replace with
Patient Books & Pays — Patient selects your center, confirms price, and completes payment through the USRad platform.	Assignment Funded — The patient or their employer completes funding through the USRad platform. Funding is confirmed before the assignment is routed.

1.9 Page 2 — credibility figures

Current	Replace with	Reason
30 Years / Imaging industry experience —	1994–2002 / Ancicare — founded,	Conflates career span with company operating life. The dates are the fact

Current	Replace with	Reason
AnciCare founded 1994	operated, and sold to CorVel	
1,200+ / Imaging centers successfully recruited — \$180M+ in volume	1,200+ / Imaging centers recruited — over \$150 million in provider volume	Center count is documented and stays precise. Volume is extrapolated; round down. Also resolves the \$180M vs TrustBar's \$160M conflict
168,000+ / Patients served across our prior imaging network	150,000+ / Patients served across our prior imaging network	Five significant figures on an extrapolation. Rounded down, no less impressive, defensible under questioning

Site conforming changes: TrustBar.astro:77-80 (\$160M+), ProvenSuccess.astro:17-29 (\$180M), AncicareLegacy.astro:96 , CredibilityBar.astro:26 , FinalCTA.astro:103 — all must state the same rounded figures.

1.10 Page 1 — layout

Trailing whitespace measures approximately 21%, not the 40% previously stated. Correction noted.

At 21% this is noticeable but not broken. If tightened, the fix is distributing space through the four sections above and moving the CTA block toward the bottom margin — not adding content. **Lowest priority item in this spec.**

2. EMPLOYER ROI REPORT

Generated template via /api/employer-roi-report . All changes are code. **Execute as a single pass** — Release 1 and Release 2 items together, since the compliance fixes and the rate language describe the same commercial terms and splitting them ships an incoherent intermediate.

2.1 Rate language — pages 1, 2, 3

Current	Replace with
USRad flat rates (\$350)	USRad network rates
at \$350 flat rate /scan	at network rates, varies by market

The document already contradicts itself: page 3's table shows MRI at \$260–350 and CT at \$250–350. **The table is correct.** Make the headline consistent with the table already present in the same document.

| 2.2 Compliance block — page 5, replace in full

Current	Replace with
ERISA Compliant — Structured as a permissible supplemental benefit — no plan amendments required	Plan Structure — Designed as a supplemental benefit that operates alongside your existing plan. Your benefits counsel should confirm fit with your plan documents
HIPAA / SOC 2 Type II — Full data security certification maintained	Data Security — HIPAA-compliant PHI handling protocols and Business Associate Agreements with all network participants
Licensed All 50 States — Workers' comp imaging approved in every jurisdiction	Network Coverage — Contracted imaging centers across [N] states, all facility-accredited and independently licensed
TPA Integration — Works with Sedgwick, Gallagher Bassett, ESIS, Broadspire + all major TPAs	Direct Contracting — USRad contracts directly with your organization. No carrier or TPA intermediary required
Zero Network Conflicts — Supplemental overlay	Keep

Four changes: removes a legal conclusion, removes a certification that does not exist, removes a jurisdictional approval claim for a deferred product, and replaces carrier positioning with direct-to-employer.

If a SOC 2 Type II report actually exists, state it separately and precisely — "SOC 2 Type II report available on request." Do not pair it with HIPAA.

| 2.3 Implementation roadmap — page 5

Week	Current	Action
1	· Execute master services agreement	Keep, and add funding bullet — see 2.4
1	· Review historical imaging spend data	Keep
1	· Map existing WC & health plan workflows	Replace: · Map covered population and eligibility data
1	· Identify implementation stakeholders	Keep

Week	Current	Action
1	· Finalize TPA integration requirements	Delete
2–3	· Configure employer portal & booking	Keep
2–3	· Coordinate with benefits platform / TPA	Delete
2–3	· Establish claims billing & reporting feeds	Replace: · Establish utilization reporting
2–3	· Complete integration testing & validation	Keep
2–3	· Activate network in employee geography	Keep
4	· Deploy employee communications	Keep
4	· Train HR team & WC adjusters	Replace: · Train HR and benefits team
4	· Activate employer dashboard	Keep
4	· Go-live with dedicated support	Keep
4	· Review early utilization baselines	Keep

| 2.4 Funding language — page 5, Week 1

Add as a new bullet under "Execute master services agreement":

	· Establish program funding arrangement
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This is the employer-side mirror of [Implementation.astro:50](#) and it is the change that makes the rest of the document coherent. Same language should be written into the page component in the same pass.

| 2.5 Savings figures — internal consistency

The document states four different savings rates about the same program.

Location	Figure
p.1, p.2, p.3 headline	85%
p.4, Day 1	50–70%
p.4, Month 3	40–60%

Location	Figure
p.5, historical	50–70%

Resolution: the headline is the computed figure and stays, **but must be qualified to the modeled comparison** — "up to 85% below the illustrated hospital outpatient baseline," not a bare 85%. Unqualified, a reader takes it as expected average savings across all imaging, which it is not.

The Day 1 and Month 3 lines must be conformed to the headline or restated as ranges that contain it. The p.5 figure is AnciCare historical and should be labeled as such.

Baseline inconsistency: the headline uses \$2,400 average hospital rate; page 3's table shows \$2,400–3,200 and the bar chart anchors at \$3,200. Pick one baseline and use it in all three places.

2.6 Unsupported claims

Location	Current	Action
p.3	"USRad is the only solution that addresses both populations"	Delete "only." Superiority claim, unsubstantiated
p.4	"Average 73% increase in necessary imaging completed"	Source or delete
p.4	"18+ day reduction in average disability duration"	Source or attribute to AnciCare experience
p.1, p.5	"Founded the managed imaging category"	Soften: "an early operator in managed imaging"

2.7 The upward-only hedge — keep, but ground it

"Actual total savings typically run 20–40% higher" appears twice with nothing pointing the other direction.

The underlying claim is sound and comes from operating experience: faster imaging means faster return to the referring physician, which means fewer indemnity days, and indemnity savings frequently exceed the imaging cost itself.

Attribute it rather than asserting it as a general rule:

Based on AnciCare client experience, indemnity and productivity savings frequently exceeded the imaging cost reduction itself. Those figures are excluded here.

Also add one downward acknowledgment so the hedges are not unidirectional — actual results depend on utilization and redirection rates, which the footer disclaimer already says and which should appear near the projection rather than only at the end.

| 2.8 Contact

(888) USRad24 → (866) USRad24 . Template fix.

app.usrad.com/signup confirmed correct.

| 2.9 Workers' comp

Per D4, de-emphasize without dismantling. The WC math, the 120-scan input, and the savings column stay. Remove availability and approval claims only — the compliance block item in 2.2 and the adjuster training item in 2.3 cover this.

Add a label to the WC savings column (p.3) and the WC output tile (p.1):

Illustrative future expansion scenario — not included in the current USRad employer offering.

Live WC math sitting next to copy describing WC as future expansion is confusing to a prospect. The label resolves it at no structural cost, which is the cheapest available fix and avoids the calculator rebuild the plan has parked.

| 3. PROVIDER PAGE — ScannerUtilization

| 3.1 The revenue figure

[provider/ScannerUtilization.astro:145-150](#)

Current	Replace with
\$37,500/month * Based on average cash-pay MRI reimbursement across modality mix and U.S. markets	\$3,400–\$10,000/month * Based on 15–25 additional plain MRI scans per month at contracted per-scan reimbursement of \$225–\$400. Rates vary by market. Results vary

\$37,500 implied 100 scans/month at \$375. Under the settled economics and the 15–25 scan basis, the same tile becomes \$3,400–\$10,000.

This is a 75–90% reduction in the provider page's headline revenue claim. It follows directly from standardizing on 15–25 scans/month and from correcting the per-scan figure. Two things make it the right call: it now matches the brief, so a center reading both sees one number; and it is a floor USRad can beat, which is the underpromise posture.

| 3.2 Other ScannerUtilization items

Line	Item	Action
:418	"Pre-screened. Pre-paid."	→ "Pre-screened. Pre-funded."
:320-347	Day 0 → Day 10 two-lane timeline	Keep. True under prefunding
:346	"zero collections risk"	Keep. True for the center under PSA § 4.3
:377-380	"10-Day Disbursement Guarantee / No claim filing"	Keep under prefunded model
:405-409	"10 / Days to disbursement / Guaranteed. Every scan."	Keep

| 4. WHAT THIS SPEC DOES NOT COVER

Blocked on the flat-fee table. Only plain MRI is settled at \$75. Ultrasound, CT, X-ray, DEXA, mammography, and PET fees are open. No document may state modality-mix economics until that table exists.

Blocked on the accounting decision. Gross vs. net revenue treatment. The AnciCare precedent (\$475 revenue, \$400 COGS) is a real fact pattern, but USRad differs on price discretion — the center sets its rate and USRad adds a fixed fee. The strongest counterweight is v6.1 Exhibit A's alternate-rate approval clause, which should be preserved and actually exercised. Accountant's read required before the Series A model.

Out of scope, unchanged: all structural rebuilds, WC architecture, [AssignmentFlowDiagram](#), [ROI Calculator](#) restructuring. Per Implementation Plan v2.

Defect track: "See Full Cost Analysis" button non-functional on /employer.
[ROI Calculator.astro:593](#) WC math defect. Both documented, neither part of this repositioning.

USRad-Repositioning-Plan-v2 (1)

| USRad Repositioning — Implementation Plan v2

August 3, 2026 | Internal Document | Not for Distribution

Supersedes the six-phase plan. Restructured to three releases per advisor review.

Governing constraint: change only what is false, contradictory, or blocking. Everything not listed here stays as written.

| GOVERNING THESIS

USRad is a prefunded imaging network. Some assignments are prepaid by the patient. Some are prefunded by an employer. Either way, funding assurance is secured before the assignment is routed.

Four sentences to lock before any file is touched:

- 1. USRad is a prefunded imaging network.
- 2. Funding assurance precedes routing. Invoicing in arrears requires a documented exception.
- 3. One contracted rate per center per modality across funded lanes. Rates vary by market.
- 4. Cash-pay remains a supported lane. Self-funded employers become the primary commercial target.

No further strategic discussion is required before Release 1.

| DECISIONS

Blocking — settle now:

	Decision
D1	Funding assurance before routing is the standard. Arrears invoicing is an exception requiring approval
D2	One contracted rate per center per modality across funded lanes; rates vary by market

	Decision
D5	Cash-pay is a lane, not the company's identity

Deferrable — do not block Release 1:

	Status
D3 — partial member cost share	Interim site language: <i>"Member cost-sharing, if any, is established by the employer program and collected by USRad — not the provider."</i> Settle before the employer agreement
D4 — workers' comp	Already deferred. Only open question is how aggressively to remove it. Answer: de-emphasize, don't dismantle

| RELEASE 1 — CORRECT WHAT IS FALSE

Purpose: nothing public contradicts the prefunded thesis. No layout changes, no architecture changes.

| 1A — Provider page: prepaid vocabulary

File:line	Current	Change
provider/HeroSection.astro:61	We bring pre-paid patients.	pre-funded assignments
provider/ScannerUtilization.astro:418	Pre-screened. Pre-paid.	Pre-screened. Pre-funded.
provider/GuaranteeSection.astro:74	Pre-qualified patients who arrive.	Disambiguate — clinical or financial
provider/FAQSection.astro:369-371	patients pre-arrange their own payment	funding secured before appointment confirmed
provider/FAQSection.astro:379-381	because patients have pre-arranged payment	same substitution

[HeroSection.astro:61](#) is the highest-risk sentence on either page. False under every model, including prefunded — the employer paid, the patient did not. Three claims in the same sentence inherit the fix.

1B — Provider page: FAQ 6 rewrite

`provider/FAQSection.astro:312-333` . Self-contained `<details>` body. Currently defines USRad as cash-pay-only, asserts patient-as-payer, and promises no-claims workflow, all in a flat "No." to a compliance question.

Rewrite whole. Must hold under employer volume: prefunded network, two funding lanes, no claims under either, no overlap with the center's existing payer contracts.

1C — Employer page: remove the four claims-workflow statements

File:line	Action
<code>employer/DualSolution.astro:93-94</code>	"Imaging centers bill the program directly" — delete. Contradicts four /provider promises
<code>employer/ExecutiveFAQ.astro:224</code>	"Direct billing options for both WC and health claims" — delete
<code>employer/Implementation.astro:86</code>	"Claims workflow setup" — delete. Timeline holds at three bullets
<code>employer/IntegrationPartners.astro:63</code>	Remove "claims integration" from the three-item list

Preserve: `Implementation.astro:51` (Claims data analysis) and `ExecutiveFAQ.astro:230` (Claims data feeds). Analytics, not funding mechanics. Fully compatible with prefunding.

1D — Compliance and performance claims

Promoted into Release 1. Not caused by the repositioning, more urgent than most of it.

File:line	Claim	Issue
<code>employer/ExecutiveFAQ.astro:278</code>	licensed in all 50 states	No license type named
<code>employer/ExecutiveFAQ.astro:276</code>	HIPAA certified	No such certification exists
<code>employer/ExecutiveFAQ.astro:274</code>	ERISA compliant	Legal conclusion asserted flatly
<code>employer/ExecutiveFAQ.astro:281</code>	Workers' comp approved	Approval claim for a deferred product

File:line	Claim	Issue
employer/ExecutiveFAQ.astro:185-186	Average client saves \$1.7M	No live employer clients exist
employer/ExecutiveFAQ.astro:76-77	implemented this exact model 1,200+ times	Center-recruitment count reused as implementation count
employer/Implementation.astro:131-132	Our Implementation Never Fails / 168,000+	Absolute claim; third reuse of an AnciCare number
employer/CostAnalysis.astro:97	"Real Example: Lower Back Pain"	Undisclosed illustrative content. EmployerCaseExample models correct disclosure four times

| 1E — Workers' comp availability claims only

De-emphasize. Do not dismantle architecture.

- [employer.astro:22](#) — <title> leads with Workers' Comp
- [employer/ExecutiveFAQ.astro:281](#) — covered in 1D
- [employer/ExecutiveFAQ.astro:175-176](#) — Month 6 WC disability reference
- [employer/Implementation.astro:107](#) — "HR/adjuster training"
- [employer/FinalCTA.astro:27](#) — "One solution for both populations"

Interim treatment for the WC architecture: label it *"Future program expansion: workers' compensation and other payor-sponsored imaging programs."* Preserves [DualSolution](#)'s two-column layout, the calculator's WC input, and [IntegrationPartners](#) without representing any of it as launched.

| 1F — ProviderBrief PDF

[provider/ProviderBrief.astro:186](#) →
[/downloads/USRad_Provider_Opportunity_Brief.pdf](#)

Un-versioned, gated, explicitly marketed for internal forwarding ([:174-180](#)). Every other artifact can be changed silently. This one is already in circulation.

Review against the prefunded thesis, version the file, republish. Separate deliverable.

Release 1 outcome: no provider is told every patient personally prepays. No employer is told USRad administers claims. No unsupported compliance claim remains. No layout changed.

| RELEASE 2 — STATE THE EMPLOYER PROPOSITION

Three new ideas. Nothing more.

1. **The employer is the buyer**, not the employee.
2. **\$0 member cost is the steering mechanism**, not a benevolence message. Anything the employee pays leaves the hospital a live option.
3. **Funded account with monthly reconciliation**. The employer maintains an agreed funding arrangement before routing and receives a consolidated statement.

| 2A — Hero proposition rebuild

`employer/EmployerHero.astro:68` — "\$260 MRIs (not \$3,200)"

Budget this as thinking, not typing. Small in file count, but it is the page's core proposition. Current framing sells employee affordability; the replacement sells employer spend reduction with \$0 as the steering lever.

The supporting arithmetic already exists on the page: imaging currently landing on the plan at hospital rates, plus `IndustryData.astro:33` (67% skip, EBRI 2023), `:99` (+\$847 per delayed diagnosis), `:79` (31% longer disability). Skipped imaging is deferral, not savings.

| 2B — Member-affordability examples

- `employer/DualSolution.astro:175` — "\$260 MRIs employees can actually afford"
- `employer/CostAnalysis.astro:180` — "Day 1: Pain starts, books \$260 MRI"

| 2C — Funding language insertion

Primary: `employer/Implementation.astro:47-53`, Week 1. `:50` • `Execute master agreement` is the money moment and says nothing about money. Plain four-item `<ul>`; a fifth bullet drops in at zero layout cost.

Secondary: `employer/ExecutiveFAQ.astro` — new `<details>` between Q3 (closes `:190`) and Q4 (opens `:192`). Five executive Q&As and none asks how the employer pays.

Uniform accordion stack; a sixth entry is purely additive.

Do not use: `R0ICalculator.astro:157-165`. Sits inside an animated results grid driven by `animateValue` at `:604-611`.

| 2D — Rate labeling

Market-based pricing is the model, not a defect. The prices are unlabeled, not wrong.

File:line	Add context
<code>employer/EmployerHero.astro:68</code>	\$260
<code>employer/R0ICalculator.astro:123</code>	\$350
<code>employer/EmployerCaseExample.astro:204</code>	\$420
<code>employer/R0IStatPanel.astro:49</code>	\$640

`R0IStatPanel:69` hands off to `R0ICalculator` and they render back to back at `employer.astro:42-44`. Those two need consistent framing at minimum.

Two genuine conflicts:

- `employer/DualSolution.astro:110` — "Transparent **flat** rates" → fixed per market
- `employer/DualSolution.astro:287` — "Combined volume drives even better pricing" — volume cannot move price under D2

No longer requires change (reversing prior guidance): `MarketScopeShowcase` strategy panel, `FoundingPartners.astro:211-215`, `HowItWorks.astro:133`, `ConsultationCTA.astro:89`. Market-varying rates are the model.

| RELEASE 3 — PSA AND MINIMUM FUNDING GATE

| 3A — The narrowed PSA approach

Do not draft a broad v7.0. The advisor's question — can the existing Fully Funded architecture be generalized without introducing a second provider workflow — resolves yes.

Broaden, do not rename. v6.1 § 1.7 already reads: *"Fully Funded Assignment" means an Assignment for which USRad has secured funding prior to the provision of services.* That definition already covers employer-prefunded work; nothing in it names the patient as payer.

Keeping the term avoids:

- the `assignment_type` enum migration (`fully_funded` remains valid)
- `renderAssignmentTypeBadge()` changes
- a migration addendum for providers already executed under v6.1

The term is cosmetically awkward once Claims-Funded goes dormant. Not worth a platform migration.

The routing warranty should be deleted, not reworded. The advisor correctly caught that the proposed phrasing implied USRad owes *only* where the gate failed. But their three-part fix is redundant: if payment is unconditional on fulfillment, no carve-out is needed.

v6.1 § 4.3 already is that promise: *"For Fully Funded Assignments, USRad shall disburse Provider compensation within ten (10) business days following Fulfillment Complete."* No funding condition. Unconditional as written.

The funding gate stays as **internal routing policy**. Putting it in the PSA would create a provider breach claim for something that causes the provider no harm.

Net PSA scope:

Action	Item
Broaden	§ 1.7 — clarify funding may be secured from the member, employer, sponsor, or other approved source
Preserve	§ 4.3 payment trigger, unchanged
Deactivate	Dormant Claims-Funded requirements that do not apply
Fix	The v6.1 defect list below
Add	Anti-circumvention; BAA mechanism

No Article IV structural change. No new Assignment Types.

| 3B — v6.1 defects (independent of the repositioning)

Defect	Location
§ 4.1 "regardless of Assignment type" contradicts Exhibit A "exclusively to Fully Funded"	Article IV / Exhibit A
§ 4.5 Member Non-Collection absent from survivability	§ 9.1
§ 2.7 requires electronic claims for work generating no claim	§ 2.7
No BAA requirement anywhere	Article II
No anti-circumvention provision	Article VI or new

Defect	Location
Exhibit A "Program Expansion" overrides Article VIII negative consent	Exhibit A

Anti-circumvention becomes urgent the moment USRad introduces a center to an employer.

| 3C — Minimum Funding Gate

Operational definition, required before engineering, operations, counsel, and sales can use the term consistently:

Funding assurance means an active and approved funding arrangement sufficient to support routing under USRad policy. It may be satisfied by secured patient payment, an employer-funded account with sufficient available balance, assignment-specific prefunding, or another mechanism approved by USRad.

This belongs in the Founder Decision Memo and the Funding Gate specification. It does **not** belong in the provider PSA — the provider does not need to reason about how USRad is funded.

Specify only. Do not build the employer platform.

Seven checks before routing:

1. Employer program active
2. Member eligible
3. Valid physician order
4. Approved service
5. Funding arrangement active
6. Sufficient available balance or capacity
7. Routing permitted

Balance tracking, reconciliation statements, and employer dashboards follow once an employer implementation is real.

This gate is the load-bearing build. It is what makes the contract language and the site copy true.

| EXPLICITLY OUT OF SCOPE

Cut from the immediate work order. May become real work later; none is necessary to make the current positioning truthful.

Structural rebuilds: `DualSolution` two-column architecture · full WC removal · `ROI Calculator` restructuring · `AssignmentFlowDiagram` redesign · `ReferralWorkflow` grid arity · `GuaranteeSection` grid · `ScannerUtilization` two-lane card

Platform: claims intake · 837 processing · repricing engine · TPA integration · treasury automation · employer-specific settlement variants · credit underwriting policy beyond "exceptions require approval"

Documents: full Provider Manual · Payor Assignment Addendum · Funding Schedule Tiers 3 and 4 (arrears variants are exceptions under D1, so the schedule reduces to prefunded arrangements plus an exception process)

Leave as written: `TrustBar` · `ProvenSuccess` · `ProviderSpineNav` · `NetworkBuilding` · `MarketOpportunityTeaser` · `CredibilityBar` · `IndustryData:27-86` sourced tiles · `EmployerCaseExample` disclosure apparatus · `ReferralWorkflow:318-325` · provider FAQs 1, 4, 5, 8, 9

Dead code — do not touch: `PriorityMarketsMap` (imported at `provider.astro:17`, never rendered) · `provider/Test.astro` · `ExitModal.astro.backup` · `employer/HeroEmployer.astro` · `employer/PartnerMfgGrid.astro`

Note `PartnerMfgGrid` asserts manufacturer endorsement (Siemens/GE/Philips/Canon/Hitachi). Ships nothing today, live in the tree.

Documented, not repaired: `ROI Calculator.astro:593` computes the return-to-work line as 16% of *total* savings including health-plan scans, then labels it "(WC)". Math defect with no mechanical relationship to the WC input. Real, but not part of this repositioning.

| TRACK 4 — EMPLOYER CONTRACTING ARCHITECTURE

Parallel track, not a fourth release. Established so it does not remain an indefinitely acknowledged gap while marketing and provider work proceed.

The employer agreement does not exist.

All three releases leave USRad with a truthful site and a clean PSA, and still nothing to sign an employer with. Required document set:

- Employer agreement (services, term, rates, data and PHI handling, ERISA positioning, indemnity, termination)

- Funding arrangement terms (reduced scope per D1 — prefunded plus exception process)
- Employer rate exhibit
- BAA

This is correct sequencing, since provider recruitment is the near-term goal and does not require it. But it is the item standing between USRad and revenue, and it should stay visible rather than sitting behind work that feels productive.

Open question for counsel, independent of everything above: does the intended employer contracting and money-movement structure require a certificate of authority under Fla. Stat. § 626.88? Under prefunding with no member collection and no plan funds held, exposure is materially lower than under a claims model. Confirm before the employer agreement is drafted.

| SEQUENCE

This week: lock D1, D2, D5 in a one-page founder decision memo. No file changes before that.

Release 1: 1F (PDF, already in circulation) → 1A and 1C (nine line-level edits) → 1D (compliance) → 1B (FAQ 6) → 1E (WC availability).

Release 2: 2C first (cheapest, locks prefunding publicly) → 2A → 2B → 2D.

Release 3: 3B (v6.1 defects, independent of everything) can start in parallel with Release 1. 3A and 3C after D1.

Everything about billing mechanics and carrier positioning resolves on the employer page. Everything about company narrative resolves on the provider page. Rates resolve on both. Two edit passes, different owners, parallelizable once D1 is settled.

Prepared for advisor review. Companion documents: /provider audit, /employer audit, PSA v6.1→v7.0 Change Analysis, Position Revision — Funding Gate.

USRad-PSA-Exhibit-C-BAA-DRAFT (1)

| EXHIBIT C: BUSINESS ASSOCIATE AGREEMENT

Structural note. Drafted as an exhibit rather than woven into the PSA body. Three reasons: BAAs amend on a different cycle than commercial terms; a center's compliance officer often reviews the BAA separately from the business agreement; and if a center insists on its own form, swapping an exhibit is easier than restructuring the agreement. It executes in the same DocuSeal envelope, so there is no added friction at signing.

Items marked **[DECIDE]** are USRad-specific and are not boilerplate.

RECITALS

This Business Associate Agreement ("BAA") supplements the Provider Service Agreement between Provider ("Covered Entity") and U.S. Radiology of Florida, LLC d/b/a USRad ("Business Associate"), and is effective as of the Effective Date of that Agreement.

With respect to the functions described in Section 2.1, Business Associate performs services on behalf of Covered Entity involving Protected Health Information. The parties enter this BAA to comply with the Health Insurance Portability and Accountability Act of 1996 and its implementing regulations at 45 C.F.R. Parts 160 and 164.

The parties acknowledge that Business Associate may also perform functions on behalf of other covered entities, including group health plans, under separate business associate agreements, and that certain disclosures between the parties may occur for treatment, payment, or health care operations purposes independent of this BAA. This BAA governs only the functions described in Section 2.1.

1. DEFINITIONS

1.1 Capitalized terms not defined here have the meanings assigned in 45 C.F.R. Parts 160 and 164, including: Breach, Covered Entity, Business Associate, Designated Record Set, Disclosure, Electronic Protected Health Information, Individual, Protected Health Information, Required By Law, Secretary, Security Incident, Subcontractor, Unsecured Protected Health Information, and Use.

1.2 "HIPAA Rules" means the Privacy, Security, Breach Notification, and Enforcement Rules at 45 C.F.R. Parts 160 and 164.

1.3 "PHI" means Protected Health Information created, received, maintained, or transmitted by Business Associate on behalf of Covered Entity.

2. PERMITTED USES AND DISCLOSURES

2.1 Services. Business Associate may Use and Disclose PHI only as necessary to perform its obligations under the Agreement, including:

- (a) routing and coordinating Assignments;
- (b) confirming funding status for an Assignment;
- (c) receiving, storing, and transmitting diagnostic reports to ordering physicians and other authorized recipients;
- (d) administering payment and reconciliation;
- (e) quality assurance and utilization review; and
- (f) any other function expressly authorized by the Agreement.

2.2 Management and Administration. Business Associate may Use PHI for its own proper management and administration and to carry out its legal responsibilities. Business Associate may Disclose PHI for such purposes only if the Disclosure is Required By Law, or if Business Associate obtains reasonable assurances from the recipient that the PHI will be held confidentially, used only as permitted, and that the recipient will notify Business Associate of any breach of confidentiality.

2.3 Data Aggregation. Business Associate may Use and Disclose PHI to provide Data Aggregation services relating to the health care operations of Covered Entity, as permitted under 45 C.F.R. § 164.504(e)(2)(i)(B).

2.4 De-Identification. Business Associate may de-identify PHI in accordance with 45 C.F.R. § 164.514(a)–(c). De-identified information is not PHI and is not subject to this BAA.

Business Associate may Use de-identified information for any lawful purpose, including network analytics, capacity planning, pricing analysis, quality measurement, and product development. Business Associate may Disclose de-identified information externally only in aggregated form that does not identify Covered Entity, any Individual, any employer, or any referring entity, except with Covered Entity's prior written permission.

Resolved. Broad internal use is preserved for MarketScope, employer utilization reporting, and network analytics. External disclosure is limited to aggregate form not

naming the center. Naming a center in comparative benchmarking requires written permission. This is the formulation most likely to survive a compliance officer's review without a negotiation.

2.5 Prohibited Uses. Business Associate shall not Use or Disclose PHI other than as permitted by this BAA, the Agreement, or as Required By Law. Business Associate shall not Use or Disclose PHI in a manner that would violate Subpart E of 45 C.F.R. Part 164 if done by Covered Entity.

2.6 Minimum Necessary. Business Associate shall request, Use, and Disclose only the minimum amount of PHI necessary to accomplish the purpose of the request, Use, or Disclosure.

2.7 No Sale of PHI. Business Associate shall not directly or indirectly receive remuneration in exchange for PHI except as permitted by 45 C.F.R. § 164.502(a)(5)(ii).

2.8 Marketing. Business Associate shall not Use or Disclose PHI for marketing or fundraising purposes.

| 3. SAFEGUARDS

3.1 General. Business Associate shall use appropriate safeguards, and comply with Subpart C of 45 C.F.R. Part 164 with respect to ePHI, to prevent Use or Disclosure of PHI other than as provided by this BAA.

3.2 Security Rule Compliance. Business Associate shall comply directly with the applicable requirements of the Security Rule, including conducting and documenting a risk analysis, implementing administrative, physical, and technical safeguards, and maintaining written policies and procedures.

3.3 Workforce. Business Associate shall train members of its workforce on their obligations under this BAA and the HIPAA Rules, and shall apply appropriate sanctions for violations.

| 4. REPORTING

4.1 Unauthorized Use or Disclosure. Business Associate shall report to Covered Entity any Use or Disclosure of PHI not permitted by this BAA of which it becomes aware, without unreasonable delay and in no case later than **ten (10) business days** after discovery.

4.2 Breach Notification. [DECIDE] Business Associate shall provide Covered Entity with preliminary notice of any Breach of Unsecured PHI without unreasonable delay and in no case later than **five (5) business days** after discovery. Preliminary notice shall include the fact of the Breach and such information as is then known.

Business Associate shall supplement the preliminary notice as the investigation proceeds, and shall provide a complete account — including the Individuals affected, a description of what occurred, the types of PHI involved, and the steps taken to investigate, mitigate, and prevent recurrence — within **fifteen (15) business days** after discovery, or as soon thereafter as reasonably practicable.

Decision required — the operational half. Two-stage notice is now drafted: five business days preliminary, fifteen for the complete account. This is more provider-friendly than a single ten-day deadline and easier to actually meet, because the short clock attaches to a fact rather than to a finished investigation.

The constraint is not the deadline. **It is discovery.** The clock runs from when Business Associate knew or reasonably should have known. Without monitoring and alerting capable of surfacing an incident, a five-day commitment cannot be met regardless of what the document says. Confirm detection capability before agreeing to five.

The proposed Security Rule update contemplates a 72-hour standard for certain incidents. If adopted, this provision would need conforming.

4.3 Security Incidents. Business Associate shall report Security Incidents as required by 45 C.F.R. § 164.314(a)(2)(i)(C). The parties acknowledge that unsuccessful attempts that do not result in unauthorized access — including routine port scans, pings, and blocked login attempts — need not be reported individually, and that periodic aggregate reporting satisfies this Section as to such attempts.

4.4 Mitigation. Business Associate shall mitigate, to the extent practicable, any harmful effect known to it of a Use or Disclosure in violation of this BAA.

5. SUBCONTRACTORS

5.1 Business Associate shall ensure that any Subcontractor that creates, receives, maintains, or transmits PHI on its behalf agrees in writing to restrictions and conditions at least as restrictive as those that apply to Business Associate under this BAA, in accordance with 45 C.F.R. §§ 164.502(e)(1)(ii) and 164.308(b)(2).

5.2 [DECIDE] Business Associate shall maintain a current list of Subcontractors with access to PHI and make it available to Covered Entity upon reasonable request.

Decision required. This is standard and increasingly requested. Before agreeing, confirm the downstream chain is actually papered: cloud and database hosting, email and messaging, document execution, analytics, monitoring and error logging, any AI or LLM vendor processing PHI, and any teleradiology or transcription vendor. Each requires its own executed BAA. **This is the item most likely to be incomplete at go-live** — vendor BAAs are easy to overlook and slow to obtain.

| 6. INDIVIDUAL RIGHTS

6.1 Access. Business Associate shall make PHI in a Designated Record Set available to Covered Entity, or as directed to an Individual, within **fifteen (15) business days** of request, to enable Covered Entity to meet its obligations under 45 C.F.R. § 164.524.

6.2 Amendment. Business Associate shall make PHI in a Designated Record Set available for amendment and incorporate amendments as directed by Covered Entity, within **fifteen (15) business days** of request, in accordance with 45 C.F.R. § 164.526.

6.3 Accounting of Disclosures. Business Associate shall maintain and make available the information required to provide an accounting of Disclosures within **twenty (20) business days** of request, in accordance with 45 C.F.R. § 164.528.

6.4 Restrictions. Business Associate shall comply with any restriction on Use or Disclosure of PHI that Covered Entity has agreed to under 45 C.F.R. § 164.522, provided Covered Entity notifies Business Associate of the restriction in writing.

6.5 Requests Received Directly. If Business Associate receives a request directly from an Individual under Sections 6.1 through 6.4, it shall forward the request to Covered Entity within **five (5) business days** and shall not respond directly unless directed to do so by Covered Entity.

Operational prerequisite — not a drafting issue. Sections 6.1 through 6.5 create hard deadlines. Before execution, USRad needs a documented intake path: where requests arrive, who verifies the requester's identity, who owns the response, where deadlines are tracked, and how Covered Entity instructions are recorded. The language is standard; the queue behind it does not exist yet. Missed deadlines here are a compliance failure regardless of good faith.

| 7. GOVERNMENT ACCESS

Business Associate shall make its internal practices, books, and records relating to the Use and Disclosure of PHI available to the Secretary for purposes of determining compliance with the HIPAA Rules. Business Associate shall notify Covered Entity of such a request unless prohibited by law.

| 8. OBLIGATIONS OF COVERED ENTITY

8.1 Covered Entity shall notify Business Associate of any limitation in its notice of privacy practices, any change in or revocation of an Individual's permission to Use or Disclose PHI, and any restriction agreed to under 45 C.F.R. § 164.522, to the extent any of these affects Business Associate's permitted Uses or Disclosures.

8.2 Covered Entity shall not request that Business Associate Use or Disclose PHI in a manner that would not be permissible under the HIPAA Rules if done by Covered Entity, except as permitted under Sections 2.2 through 2.4.

8.3 Covered Entity represents that it has obtained any consents, authorizations, or permissions necessary for Business Associate to perform its obligations under the Agreement.

| 9. TERM AND TERMINATION

9.1 Term. This BAA is effective as of the Effective Date of the Agreement and terminates when all PHI is returned or destroyed under Section 9.4, or when the protections of Section 9.4(c) are extended.

9.2 Termination for Cause. Covered Entity may terminate this BAA and the Agreement if Business Associate materially breaches this BAA and fails to cure within **thirty (30) days** of written notice. Business Associate has the same right with respect to Covered Entity.

9.3 Automatic Termination. This BAA terminates automatically upon termination of the Agreement, subject to Section 9.4.

9.4 Effect of Termination. [DECIDE] Upon termination:

(a) Business Associate shall return or destroy all PHI it maintains in any form, and shall retain no copies, to the extent feasible;

(b) Business Associate shall ensure its Subcontractors do the same; and

(c) where return or destruction is not feasible — including where retention is required by law, by professional record retention standards, or for audit, dispute resolution, or payment reconciliation purposes — Business Associate shall extend the protections of this BAA to that PHI and limit further Uses and Disclosures to the purposes making return or destruction infeasible, for so long as it is retained.

Decision required. Subsection (c) is where most of the practical answer lives. Diagnostic reports carry state retention requirements, and USRad needs transaction records for audit and reconciliation under PSA § 4.11. Confirm the applicable Florida retention period and make sure the platform's data lifecycle actually implements whatever this says — a retention clause the system does not honor is worse than no clause.

| 10. MISCELLANEOUS

10.1 Regulatory Changes. The parties shall amend this BAA as necessary to comply with amendments to the HIPAA Rules or other applicable law. Either party may propose an amendment; the parties shall negotiate in good faith.

10.2 Interpretation. Ambiguities shall be resolved in favor of a meaning that permits compliance with the HIPAA Rules. In the event of conflict between this BAA and the Agreement with respect to PHI, this BAA controls.

10.3 No Third-Party Beneficiaries. Nothing in this BAA confers rights on any person other than the parties and their respective successors and assigns.

10.4 Survival. Sections 4, 6, 7, 9.4, and 10 survive termination.

10.5 Independent Obligations. Nothing in this BAA limits either party's independent obligations under the HIPAA Rules.

| SIGNATURE

Executed as part of, and subject to, the Provider Service Agreement.

COVERED ENTITY (PROVIDER)

Legal Name: ____

Signature: ____

Printed Name: _____

Title: _____

Date: _____

BUSINESS ASSOCIATE

U.S. RADIOLOGY OF FLORIDA, LLC D/B/A USRad

Signature: _____

Name: Michael Cabrera

Title: President & CEO

Date: _____

| CONFORMING CHANGES TO THE PSA

Three changes required. None is optional if this exhibit is added.

1. § 2.5 — Confidentiality and Security. Currently requires HIPAA compliance but never requires execution of a BAA. Add:

The parties shall execute the Business Associate Agreement attached as Exhibit C, which forms an integral part of this Agreement.

2. § 9.1 — Survivability. Add Exhibit C to the list of surviving provisions.

3. Exhibit lettering. Current PSA has Exhibit A (Reimbursement Schedule) and Exhibit B (Provider Locations). This becomes Exhibit C. Update any cross-references.

| WHAT THIS COMMITS USRad TO

The document is cheap. Performance is not. Signing this makes USRad directly liable under the Security Rule for:

Obligation	Status
Documented risk analysis	Required before first PHI receipt
Written security policies and procedures	Required

Obligation	Status
Workforce HIPAA training and sanctions policy	Required
Incident response plan capable of 10-business-day breach notice	Required
Executed BAAs with every downstream vendor touching PHI	Most likely gap at go-live
Access controls, audit logging, encryption	Current rule: encryption is "addressable." Proposed rule would mandate it
Designated security official	Required
Individual access / amendment / accounting request queue	Required by §§ 6.1–6.5. Does not exist yet
Monitoring capable of supporting a five-business-day discovery clock	Required by § 4.2

The proposed Security Rule update — mandatory encryption, MFA, 72-hour incident reporting, annual penetration testing, expanded business associate oversight — remains proposed, with final action currently targeted for July 2027 and no certainty it will be adopted as drafted. Building toward it now is materially cheaper than retrofitting a live platform.

| EMPLOYER SIDE — SEPARATE AND NOT COVERED HERE

A self-funded group health plan is itself a Covered Entity. Employer arrangements involving eligibility files, claims or utilization data, or member reporting require their own BAA, with the Plan as Covered Entity and USRad as Business Associate.

This is why § 2.1(b) was narrowed. Eligibility verification against a plan is performed on behalf of the plan, not on behalf of the imaging center. Placing it in the provider BAA mischaracterized the relationship. The provider BAA now covers only funding-status confirmation for a routed Assignment; plan-side eligibility belongs in the employer BAA.

Counsel should validate role assignment per data flow rather than assume one universal relationship. Depending on the transaction, USRad may be a business associate of the imaging center, a business associate of a group health plan, a recipient of information directly from an Individual, or a party to a treatment disclosure requiring no BAA at all. The four can occur within a single assignment.

That document belongs in the employer agreement set, which does not yet exist. Recorded here as a gap, not addressed by this exhibit.

Draft prepared for review by healthcare counsel licensed in the State of Florida. Not for execution. This is a legally required document carrying direct statutory liability; counsel review is not optional.

USRad-Project-Report-Strategic-Repositioning (1)
| USRad Strategic Repositioning — Project Report

August 3, 2026 | Internal Document | Not for Distribution

Prepared for advisor review. Covers the self-funded employer repositioning, the contract architecture decisions arising from it, the public-facing artifact corrections required, and the HIPAA gap identified in the course of the work.

Companion documents: PSA Exhibit C (Business Associate Agreement, draft) and Implementation Plan v2 with Addenda A and B.

| 1. EXECUTIVE SUMMARY

USRad is shifting its primary commercial target from uninsured cash-pay patients to self-funded employers. The shift is additive rather than a pivot — the cash-pay lane is retained — but it surfaced a set of contract, marketing, and compliance issues that had been latent under the original single-payer-type assumption.

Four decisions define the outcome:

	Decision
Funding standard	Funding assurance precedes routing. Employer arrears invoicing is an exception requiring approval, not the default
Rate structure	One contracted rate per center per modality across all funding lanes. Rates vary by market. USRad's fee moves from cost-plus-with-cap to flat per modality
Cash-pay posture	Retained as a lane, retired as a company identity
Contract scope	Broaden the existing Fully Funded Assignment definition rather than introducing new assignment types

The governing thesis, adopted as a standing principle:

USRad is a prefunded imaging network. Some assignments are prepaid by the patient. Some are prefunded by an employer. Either way, funding assurance is secured before the assignment is routed.

The most significant finding outside the repositioning itself: the Provider Service Agreement requires HIPAA compliance but never requires execution of a Business Associate Agreement. No BAA exists today.

Verified status: **no PSA has been executed. One center began onboarding but did not execute. No PHI has been transmitted.** This is therefore a pre-launch gap rather than an active compliance failure. It is also a hard prerequisite for network recruitment, since a center's compliance officer will not transmit PHI without an executed BAA.

The onboarding that began without a BAA available is worth noting as a near-miss. Nothing moved, but the gap is reachable through the existing flow rather than theoretical, and the remedy should be in place before onboarding resumes.

| 2. WHAT CHANGED, AND WHY

| 2.1 The market shift

USRad launched against the cash-pay segment: uninsured and underinsured patients purchasing imaging at transparent prices. That market is real and remains the base.

Self-funded employers offer materially better unit economics on the same infrastructure:

	Cash-pay	Self-funded employer
Revenue pattern	Transactional, per-patient	Recurring, contracted, population-based
Deal size	One study	An employee population
Acquisition cost	Per patient, ongoing	Per employer, amortized
Channel	Direct-to-consumer	Benefits advisors and TPAs

The competitive set is already there. Green Imaging, the clearest market reference, states that it serves over 1,500 employer health plans across roughly 5,000 facilities.¹

¹ As published by Green Imaging on its own website, August 2026. Not independently verified. Competitor-scale claims change; treat as directional.

| 2.2 The mechanic that matters

Employers commonly cover imaging at 100% with zero member out-of-pocket. This is deliberate plan design. The employer saves enough against the hospital-based allowed amount that waiving deductible and coinsurance still produces a large net saving, and the \$0 price is what actually steers the employee away from a hospital outpatient facility.

This is the fact that broke the original model. USRad's platform, contracts, and marketing all assumed the patient prepays. Under employer sponsorship there is no member transaction at the point of service, so funding must come from the employer — either before routing or after.

That question did not exist under the original design. It was created by the shift.

| 2.3 The finding that narrowed the problem

Research into how the market leader actually operates changed the scope materially.

Green Imaging's flagship employer product engineers the claim out entirely: one monthly invoice to the employer, no claims to manage, a member voucher presented at the appointment. They position it as a bolt-on benefit that coexists with existing coverage rather than a plan-integrated carve-out.

The implication for USRad: **for employers that accept the bolt-on prefunded structure, there is no new provider workflow.** The imaging center uploads a report and is paid, exactly as in cash-pay. No 837, no repricing engine, no TPA integration, no accumulator or stop-loss plumbing.

This is conditional, and the condition is real. Employers that require imaging spend to credit member accumulators or count toward stop-loss will need plan integration, which reintroduces the claim. Larger employers with heavy claims experience care about this more than small ones, which is likely why the market leader supports both paths while leading with the simpler one.

For employers that accept the bolt-on structure, this collapsed roughly 80% of the platform build and contract rewrite the earlier analysis had assumed necessary. See assumption 8.3.

| 3. CONTRACT ARCHITECTURE

| 3.1 The question that was actually at stake

Prior PSA versions carried a pay-when-paid clause for claims-funded work:

v6.1 § 4.4: USRad shall disburse Provider compensation within ten (10) business days following USRad's receipt of corresponding payor funds. USRad shall have no obligation to disburse amounts not received from the payor.

The initial recommendation was to replace this with an unconditional USRad payment obligation, on the reasoning that self-funded employers are not regulated insurers and a provider will not price that credit the way it priced a workers' compensation carrier.

That recommendation was superseded by a correction from the founder. At AnciCare, across 40,000+ cases annually, the pay-when-paid clause never produced a dispute — because authorization preceded routing and authorization was reliable. The contract clause was a backstop on a risk already eliminated operationally.

The control was the intake gate, not the contract language.

| 3.2 The resolution

This reframes the question from *how do we allocate the risk* to *how do we prevent the event*.

Under a funding-assurance-before-routing standard, the employer has prefunded or is inside an active funding cycle before any assignment reaches a center. No credit is being extended. There is nothing to guarantee against.

The provider-facing statement is operational rather than financial, and stronger for it:
USRad does not route an assignment until funding is secured.

| 3.3 The narrowed PSA scope

Following advisor review, a broad Version 7.0 rewrite was rejected in favor of a minimal amendment.

v6.1 § 1.7 already reads: *"Fully Funded Assignment" means an Assignment for which USRad has secured funding prior to the provision of services.* Nothing in that definition names the patient as payer. It already covers employer-prefunded work.

Broaden, do not rename. Retaining the term avoids an `assignment_type` enum migration, portal badge changes, and a migration addendum for any provider already executed under v6.1.

A proposed routing-gate warranty was also **deleted rather than reworded**, on the advisor's observation that the drafted phrasing implied USRad owed only where the gate failed. On review, the warranty was unnecessary: v6.1 § 4.3 already states an unconditional payment

obligation on Fulfillment Complete. The funding gate remains internal routing policy. Placing it in the PSA would create a provider breach claim for something causing the provider no harm.

Resulting scope:

Action	Item
Broaden	§ 1.7 — clarify funding may be secured from member, employer, sponsor, or other approved source
Preserve	§ 4.3 payment trigger, unchanged
Deactivate	Dormant claims-funded requirements
Fix	The v6.1 defect list (§ 3.4)
Add	Anti-circumvention; Exhibit C (BAA)

No Article IV structural change. No new assignment types.

3.4 Pre-existing v6.1 defects

Present in the current agreement, independent of the repositioning.

Defect	Location
§ 4.1 states rates apply "regardless of Assignment type"; Exhibit A states they apply "exclusively to Fully Funded Assignments." Both cannot be true	Article IV / Exhibit A
§ 4.5 Member Non-Collection absent from survivability list	§ 9.1
§ 2.7 requires electronic claim submission for work that generates no claim	§ 2.7
No BAA requirement anywhere in the agreement	Article II
No anti-circumvention provision	Article VI or new
Exhibit A "Program Expansion" clause overrides Article VIII negative consent	Exhibit A

Anti-circumvention becomes material the moment USRad introduces a center to an employer.

4. THE HIPAA GAP

| 4.1 The finding

PSA § 2.5 requires the provider to maintain PHI confidentiality in accordance with HIPAA. The agreement never requires the parties to execute a Business Associate Agreement, and none exists.

USRad receives patient identity, physician orders, exam types, and signed radiology reports, and performs assignment coordination, report routing, and payment administration on behalf of contracted centers. That is business associate activity.

Verified status: no PSA executed; one center began onboarding without completing execution; no PHI transmitted. This is a pre-launch gap, not an active compliance failure.

The distinction matters for sequencing. Because nothing has moved, USRad can put the BAA, the vendor chain, and the underlying security program in place before the first disclosure rather than remediating afterward — an advantage that disappears the moment onboarding resumes.

| 4.2 Why it is a recruitment blocker

A center with a functioning HIPAA program will not transmit PHI without an executed BAA, and will ask for one during onboarding. Absent a document to present, onboarding stalls at the point where the commercial argument has already been won.

| 4.3 The remedy

A BAA has been drafted as **Exhibit C to the PSA** (attached separately). Structural decisions:

- **Exhibit, not body text.** BAAs amend on a different cycle than commercial terms, and a center's compliance officer often reviews the BAA separately from the business agreement.
- **One signature.** Exhibits form part of the agreement they attach to; PSA § 9.3 already states this. The provider signs once, in the same DocuSeal envelope.
- **Two conforming PSA changes required:** § 2.5 must reference Exhibit C; § 9.1 must add it to survivability.

Four provisions require decisions before execution, flagged in the draft:

Provision	Decision
§ 2.4 De-identification	Load-bearing for MarketScope, employer utilization reporting, and network benchmarking. Some compliance officers push back; a

Provision	Decision
	common compromise limits disclosure to aggregate form not naming the center
§ 4.2 Breach notification	Ten business days drafted. Five is more provider-friendly and may be requested
§ 5.2 Subcontractor list	Standard and increasingly requested. Requires the downstream vendor chain to actually be papered
§ 9.4(c) Retention	Must match what the platform's data lifecycle actually implements

4.4 The obligations behind the document

The drafting is inexpensive. Performance is not. Execution commits USRad to direct Security Rule liability:

- Documented risk analysis
- Written security policies and procedures
- Workforce training and sanctions policy
- Designated security official
- Incident response capable of meeting the notification deadline
- Executed BAAs with every downstream vendor touching PHI
- Access, amendment, and accounting request handling within stated windows

Recommended approach: one-time consultant engagement for the initial risk analysis and policy set; a compliance platform for training records and vendor tracking; everything ongoing in-house. Full outsourcing is not justified at current scale.

USRad's PHI footprint is narrow — assignment data, orders, reports, payment records. Not an EHR, not a clearinghouse, not storing images at scale. That keeps the risk analysis scope genuinely small.

4.5 The vendor BAA inventory — most likely gap at launch

Infrastructure is the obvious layer: Supabase (database, ePHI at rest) and Vercel (PHI in transit, request logs) both require BAAs, and both likely gate them behind a paid tier.

Authorize.Net is a useful boundary case. Payment processors generally argue they handle financial data under PCI DSS rather than PHI. That holds only if no clinical detail is passed

in transaction metadata — an internal assignment ID rather than a procedure description keeps them out of scope.

The commonly missed categories are not infrastructure:

Category	Exposure
Transactional email and SMS	PHI in message bodies and retained logs
Error monitoring	Captures request payloads on exception
Analytics and session replay	Records form fields as typed
Support tooling	Patients paste clinical detail into chats
AI/LLM vendors	Most consumer tiers explicitly disallow PHI
Backups	Follow the data

Two architectural moves available because nothing is live: scrub PHI before it reaches optional vendors (strip request bodies from error monitoring, disable session replay on authenticated routes), and design notifications that carry no PHI ("details in your portal" rather than naming the study). Every vendor kept out of scope is one fewer BAA to obtain and audit.

Employer side, not covered by Exhibit C: a self-funded group health plan is itself a covered entity. Employer arrangements involving eligibility files or utilization data require their own BAA, belonging to the employer agreement document set.

| 5. PUBLIC-FACING ARTIFACT AUDIT

Full audits of the provider and employer pages, and both downloadable PDFs, were conducted. Findings are detailed in Implementation Plan v2 and its addenda. Summary of what matters at advisor level:

| 5.1 The two pages contradicted each other

The provider page promised "no claim filing" in four places. The employer page told employers "imaging centers bill the program directly." Both were live. A center's CFO reading both before signing would find the conflict.

Notably, the employer page stated **no funding timing at all** across thirteen components — no deposit, invoice, net terms, or funding cadence. The exposure was one-directional: the provider page over-promised into a gap the employer page left empty. That gap is where the prefunding standard gets stated, at no cost, because nothing has to be retracted.

| 5.2 Unit economics did not reconcile

The provider brief claimed **\$375 net margin per scan**. The employer ROI report showed **\$350 per scan** paid to USRad. As written, the spread was negative.

Root cause: both documents applied an MRI-tier figure and labeled it a modality-mix average. The ROI report's own scan-level table showed MRI at \$260–350, CT at \$250–350, ultrasound at \$150–250, X-ray at \$75–150. A genuine blended average lands nowhere near either headline number.

Reconciled:

	Value
Center per-scan reimbursement, plain MRI	\$225–\$400, varies by market
USRad fee, flat per modality	\$75 (plain MRI)
Published global price	\$300–\$475
Volume basis	15–25 additional scans/month
Realistic monthly add to center	\$3,400–\$10,000

Two consequential corrections follow. The provider page's headline revenue figure drops from **\$37,500/month to \$3,400–\$10,000** — a 75–90% reduction, following from standardizing the volume basis and correcting the per-scan figure. And "net margin" is replaced with "per-scan reimbursement," since USRad cannot know a center's margin.

The incremental-margin argument the \$375 figure was reaching for is retained, in prose where it cannot be arithmetically falsified: *fixed costs are already covered; volume into an unfilled slot carries only the read fee and supplies.*

| 5.3 Compliance claims requiring removal

Present on the employer page and in the generated ROI report, addressed to executives whose counsel will verify them:

Claim	Issue
"Licensed in all 50 states"	No license type named; unsupportable
"HIPAA certified — SOC 2 Type II compliance"	HIPAA certification does not exist. No SOC 2 report exists
"ERISA compliant — structured as permissible benefit"	Legal conclusion asserted flatly

Claim	Issue
"Workers' comp approved"	Regulatory approval claim for a deferred product
"Average client saves \$1.7M in year one"	Asserts live employer clients; none exist
"Implemented this exact model 1,200+ times"	Center-recruitment count reused as implementation count

These were not caused by the repositioning and are not fixed by it. They have been promoted into the first release.

5.4 Historical figures

Several AnciCare figures are extrapolations presented at precision they cannot support, and two conflict across artifacts (\$180M vs \$160M for the same fact).

Recommended treatment is presentation rather than disclaimer — round down and state as approximate: "over \$150 million" in provider volume, "150,000+" patients served, and "AnciCare 1994–2002" as the company fact rather than a career-span figure. The 1,200+ center count is documented and needs no hedging.

5.5 Scope discipline

The audits identified six components requiring structural rebuilds — diagram node counts, grid geometry, calculator restructuring. **All are driven by the claims model.** Under the prefunding standard, none is necessary. They have been explicitly parked.

6. IMPLEMENTATION STRUCTURE

Three releases, per advisor recommendation.

Release	Purpose	Contents
1	Correct what is false	Prepaid vocabulary; four claims-workflow statements; compliance claims; WC availability claims; both PDFs
2	State the employer proposition	Employer as buyer; \$0 as steering mechanism; funding language; rate labeling
3	Align contract and platform	PSA amendment; Exhibit C; v6.1 defects; minimum funding gate specification

Governing constraint: change only what is false, contradictory, or blocking.

Sequencing note: the employer ROI report is generated code, not a static PDF. Its Release 1 and Release 2 changes should be executed in a single pass, since the compliance corrections and the rate language describe the same commercial terms.

| 7. OPEN ITEMS

| 7.1 Requiring decision

Item	Owner
Flat fee table for remaining modalities (plain MRI settled at \$75)	Founder
Gross vs. net revenue treatment	Accountant
Partial member cost share — supported or 0%/100% only	Founder
Exhibit C decisions: de-identification scope, breach timing, subcontractor list, retention	Founder + counsel

| 7.2 Requiring counsel

Item	Note
Exhibit C review	Legally required document with direct statutory liability
Fla. Stat. § 626.88 administrator licensure	Whether the intended employer money-movement structure requires a certificate of authority. Materially lower exposure under prefunding with no member collection and no plan funds held than under a claims model
Compliance claim replacements	Particularly the plan-structure and data-security language

| 7.3 Track 4 — Employer Contracting Architecture

The employer agreement does not exist. All three releases leave USRad with a truthful site, a clean PSA, and an executable BAA — and nothing to sign an employer with.

Established as a **parallel track rather than a fourth release**, so it does not remain an indefinitely acknowledged gap while marketing and provider work proceed. Document set:

- Employer Services Agreement
- Funding Arrangement
- Employer Rate Exhibit

- Employer-side BAA (Plan as Covered Entity)

Work begins with an outline rather than full legal drafting, pending counsel's resolution of the money-flow structure and the § 626.88 question.

This is correct sequencing, since provider recruitment is the near-term goal and does not require it. But it is the item standing between USRad and revenue.

| 8. ASSUMPTIONS THAT MAY BE WRONG

Listed for challenge.

8.1 That employers will accept prefunding. No employer has been asked. If the market insists on arrears invoicing, the funding standard and the provider promise both need revisiting.

8.2 That providers will accept the terms. No provider has been recruited. The payment structure may not be the binding constraint on recruitment — rate may be.

8.3 That the bolt-on structure is available. Some employers will require plan integration for accumulator and stop-loss credit. That reintroduces the claim and with it the platform build. **This is the assumption on which the largest single scope reduction in this package rests.** It should be tested in the first employer or advisor conversation, with one question: does imaging need to run through the plan document, or can it be purchased alongside it?

8.4 That the corrected revenue figures still recruit. Dropping the provider page headline from \$37,500 to \$3,400–\$10,000 monthly is a large reduction in the stated opportunity. It is defensible and beatable, but it is untested as a recruitment message.

8.5 That the self-funded employer market is the right target at current network scale. Employers ask about coverage density early, and the network is small.

| Summary judgment

The repositioning is sound and the work required is smaller than initially assessed, principally because the market leader's own structure suggests that for employers accepting a bolt-on prefunded arrangement, the claim can be engineered out rather than built for. Where an employer requires plan integration, the claim returns — and so does the build.

Two items are genuinely urgent and neither is caused by the repositioning: the unsupported compliance claims currently published to executives, and the absent BAA that blocks

provider recruitment.

Prepared for advisor review. Attachments: PSA Exhibit C (Business Associate Agreement, draft); Implementation Plan v2 with Addenda A and B.